**جوستاف روسي انترناشيونال - مصر (EPE) *** **طلب علاج أول مرة / متردد**

**إسم المريض رباعياً :** سهير سلامة عبد المجيد احمد
**المحافظة :** الجيزه **الحي / المدينة :** كرداسه **العنوان :** الجيزة
**رقم البطاقة :** 27902112102367 **رقم التليفون :** 01129387118 **تاريخ تسجيل الإستمارة :** 17-10-2026
**العمر(تاريخ الميلاد) :** 11-02-1979 **المهنة :** ربه منزل

## الإقرار

**أقر انا الموقع أدناه انني لا اتمتع باي نظام علاج مثل:-التأمين الصحي-القوات المسلحه-الشرطة-الهيئات-المؤسسات او اي نظام يتعارض مع نظام العلاج علي نفقة الدولة- وسوف اتحمل كافة نفقات العلاج في حالة ثبوت غير ذلك**

**الإخصائي الاجتماعي** **المقر بما فيه**
**التوقيع:**(----------) **التوقيع:**(----------)

**التشخيص الإكلينيكي :** سرطان بالثدي (مبادرة)
**نتائج الأبحاث :** (مرفق)
**التشخيص النهائي :** سرطان بالثدي (مبادرة)
**خطة العلاج :** أبحاث أورام مزيد من الابحاث و الاشعات [7954] **قسم العلاج :** الاورام
**التعليق على التشخيص :** ----------

## تقرير اللجنة الثلاثية المتخصصة

**الاسم:** د/محمد كيلاني **الوظيفة:** ---------- **التوقيع:** ----------
**الاسم:** د/ندى حسن **الوظيفة:** ---------- **التوقيع:** ----------
**الاسم:** د/نرمين رمضان **الوظيفة:** ---------- **التوقيع:** ----------

**أطلعت اللجنة علي لائحة العلاج الخاصة بالمجالس وتلتزم جهة العلاج بلائحة المجالس الطبية.**
**كما تقر اللجنة بأن البيانات المدونة فى هذا النموذج صحيحة.**

**تعتمد صحة البيانات المرفقة**
**رئيس اللجنة و مدير المستشفي.**

**تتعهد المستشفى أن لا يتم تنفيذ القرار إلا بعد مراجعة الملف الطبي للمريض وإعادة مناظرة المريض بالمستشفي قبل الصرف.**

## أعضاء اللجنة

**ا.د/** ---------- **ا.د/** ---------- **د/** ---------- **د/** ----------

## قرار اللجنة

**التشخيص النهائي :** ----------
**خطة العلاج :** ----------
**المستشفي :** ----------**قسم :** ----------
**جهة العلاج :** ----------**نفقات :** ----------
**ملاحظات :** ----------

| رئيس | مدير عام | ختم |
|---|---|---|
| المجلس الطبي المتخصص | مديرية الشئون الصحية | شعار الجمهورية |

**Name:** سهير سلامة عبد المجيد احمد
**National ID:** 27902112102367

# تقرير طبي

**السيد**

**تحية طيبة وبعد,,**

| | |
|---|---|
| **إيماء للطلب المقدم من** | **بشأن تقرير طبي للمريض** |
| **بقسم:** | **رقم طبي:** |

**نتشرف بأن نوضح فيما يلي تقريرا عن حالته**

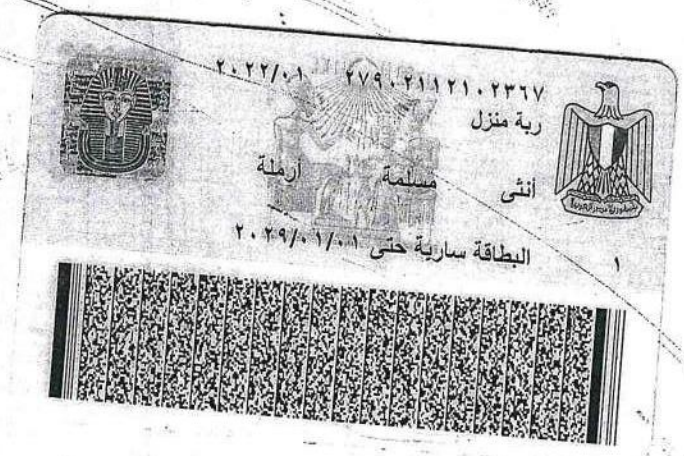
| | |
|---|---|
| **تاريخ وساعة الدخول :** -- --:-- yyyy-mm-dd | **تاريخ وساعة الخروج :** PM 01:17 05-07-2026 |

**وبعد الكشف والفحوصات والمتابعة وجد انة**

المريضة تعاني من ورم غير حميد بالثدى و تحتاج الي **Further investigations and imaging**

| | |
|---|---|
| **التاريخ :** **05-07-2026** | **الوقت :** **-- --:--** |
| | **الطبيب المعالج :** د/شيماء لاشين |




National ID · Front & Back
[[DOCTYPE:ID_FRONT_BACK]]

7007

| | |
|---|---|
| **Patient's Name:** | **soher sa aged** |
| **Date:** | **2026-04-11** |
| **Referred by:** | Prof.Dr. **ct cap iv** |
| **Hospital:** | **Goustave Roussey International Egypt** |

**MULTISCLICE POST CONTRAST C.T. EXAMINATION OF THE CHEST, ABDOMEN AND PELVIS REVEALED;**

compared to previous study dated 2044/2025, the current study reveled

**I- CHEST:**

- Still seen right lower lobe pulmonary nodule is seen measuring 7 mm.
- Resolution of the right minimal pleural effusion
- No pulmonary masses or cavitary lesions.
- No consolidation or collapse.
- No hilar or mediastinal masses or lymph node enlargement.
- No pleural or pericardial collection.
- Normal CT appearance of the heart and great vessels.

**II- ABDOMEN AND PELVIS:**

- Enlarged liver showing homogenous parenchymal attenuation. No hepatic focal lesion or dilated biliary radicles.
- Both kidneys are normal site, average sized and showing smooth regular cortical outline and preserved parenchymal thickness and function. No stones or back pressure changes.
- Mesenteric panniculitis with multiple mesenteric lymph nodes largest measuring 17 x 8 mm.
- Normal C.T appearance of the spleen, pancreas, suprarenal glands, aorta and IVC.
- The urinary bladder shows no evidence of gross masses or diverticulae or stones.
- Normal appearance of the pelvic organs with no pelvic masses or collections.
- Few subcentimeteric bilateral external iliac lymph nodes largest measuring 22 x 8 mm at the right external iliac region.
- No ascites.
- Still seen widespread multiple mixed lytic& sclerotic lesions in the axial& appendicular skeleton

**Opinion:**

❖ **Stationary course**

*Dr.sherif Wagdy*

CT Scan
[[DOCTYPE:CT_SCAN]]

| | |
|---|---|
| **Name:** سهير سلامه عبدالمجيد | |
| **MR. :** 27 | |

Detailed Medical Report

**السيد**

**تحية طيبة وبعد,,**

| | | | |
|---|---|---|---|
| **إيماء للطلب المقدم من** | | **بشأن تقرير طبي للمريض** | |
| **بقسم:** | | **رقم طبي:** | |

**ننشرف بأن نوضح فيما يلي تقريرا عن حالته**

| | | | |
|---|---|---|---|
| **تاريخ وساعة الدخول :** | | **تاريخ وساعة الخروج :** | |

**وبعد الكشف والفحوصات والمتابعة وجد انه**

**patient on arimedex zoladex + zometa for zometa 4mg**

**prem bilateral breast cancer ct4 n1 m1 met to bone by cts initially and pleura peritoneum diagnosed in 2024 idc 2 er8/8 pr8/8 her2-ve both sidex rec zoladex arimidex from 11/2024 initial cts pleural eff ascites subcentim pul nodules multilocular ovarian cyst lytic osseous lesions bs free bs 12/2025 multiple osseous lesions cts on 10/2025 rd for new cts chest xray cc dyspnea for zoladex arimidex for 1 month**

**CT CAP 11\4\2026 >> SD ARAMIDEX + SUPP >> 3M ZOMETA TODAY AFTER LABS >> PHARMACY ZOLADEX TO BE TAKEN ON 13\6\2026**

**تحريرا في ,**

| | | | |
|---|---|---|---|
| **التاريخ :** | 19-09-2026 | **الوقت :** | |

**الطبيب المعالج :** هنا اسامه عبد الرحيم عبد البر

**يعتمد ,**

GUSTAVE ROUSSY
INTERNATIONAL
EGYPT

| | |
|---|---|
| Patient Name: Sohair Salama Abd Al Maguid | Date : 05/10/2025 |
| Exam : CT CAP+C (CT Chest Abd. Pelvis) | Age : 46 Year(s) |
| Medical ID: 3375/2024 | Pat. ID: 80337 |

**POST CONTRAST MULTI-SLICE CT SCAN OF THE CHEST, ABDOMEN & PELVIS REVEALED: -**

**COMPARISON :** Compared to previous study dated 20/04/2025

**FINDINGS :**

1. **CHEST:**

- Regressive course of the Bilateral breast heterogenous parenchymal distortion with mild skin thickening .
- Few subcentimetric bilateral axillary enlarged lymph nodes.
- Complete resolution of right pleural effusion.
- Still seen right lower lobe posterior segment pulmonary nodule measuring about 6 mm.
- No pathological hilar or mediastinal lymph nodes.
- Average cardiac size. No pericardial effusion detected.

2. **Abdomen and pelvis:**

- The liver is mild **enlarged** with homogenous CT texture. No definite focal lesions or contour irregularities.
- No intra-hepatic or extra-hepatic biliary radicles dilatation.
- Normal size & parenchymal texture of the spleen with no focal lesions seen.
- Normal size, parenchymal thickness & density of both kidneys with no stones or back-pressure changes.
- Smooth filling of the urinary bladder with no filling defects or diverticular outpouching.
- Total resolution of Ascites with still smudging of peritoneal fat and nodular small densities
- Still seen mild enlarged both adnexa.
- Still seen mild circumferential gastric wall thickening , no intraluminal masses .

3. **The scanned skeleton:**

- The scanned skeleton shows stable widespread multiple mixed lytic or sclerotic lesions in axial and appendicular skeleton

**OPINION:**

- **As compared to previous study , the current study shows overall regressive course.**

DR/Reeham.Khader



CT Scan
[[DOCTYPE:CT_SCAN]]

NILE SCAN & LABS

مركز النيل للأشعة و التحاليل

Name : Soheir Salama AbdulMajid Ahmed
Branch : Dokki

Date : 01-12-2025 12:29
Medical No. : 263007

# $Tc^{99}m$ MDP SKELETAL SCINTIGRAPHY

## History

- A 46-year-old lady, with history of metastatic breast cancer.

## Technique

- Anterior and posterior whole body scans were acquired 2 hours post injection of the tracer.

## Findings

- Multiple foci of mildly increased tracer uptake at multi-levels of the spine, sternum and pelvic bones.
- Degenerative changes are noted at the spine and big joints.

## Impression:

- **Mild residual tracer active osseous deposits as described, for follow up**

Best regards,
Dr. Alaa Nagi, MD
Nuclear Medicine

NILE DIAGNOSTIC CENTER



Bone Scan
[DOCTYPE:BONE_SCAN]]

Presidential Program for Women's Health
Ministry of Health and Population

البرنامج الرئاسي لصحة المرأة
وزارة الصحة والسكان

# MDT form

A-Patient ID:
1- Name : سهير سلامة عبدالمجيد احمد
Sex: Female
3- Medical number: 83235124
Old Med : 3375-2024
2-ID numbers:
Patient Age : 46 Y

| | | | | | | | |
|---|---|---|---|---|---|---|---|
| Family history | Family History | Phone | 01159679310 | BMI | BMI | REASON OF MDT | |
| marital status | Marital Status | NRS Score | NRS Score | GROUP | GROUP | Date of MDT | |
| Co morbidity | Co Morbidity | Menopausal status | Menopausal Status | Initial STAGE | Initial STAGE | | |

## Clinical data

| Breast Imaging : | Mets Workup : |
|---|---|
| | cap: rt pulmonary nodules+rt pleural effusion<br>mild ascites+peritoneal nodules<br>bilateral adenxal masses<br>bone mets |

| BX | ER | PR | Her 2 Neu | SISH | Ki67 |
|---|---|---|---|---|---|
| IDC | 8/8 | 8/8 | 1 | | |
| BX COMMENT | ER COMMENT | PR COMMENT | Her 2 Neu COMMENT | SISH COMMENT | KI |

| T | N | M |
|---|---|---|
| | | 1 BONE,LUNG |

Sonomammography

PATIENT HISTORY

De-novo bilateral T4b breast cancer, IDC, BOTH sides strong HR+ve and HER2 -ve
CT CAP: rt pulmonary nodules+rt pleural effusion ,mild ascites+peritoneal nodules
bilateral BULKY adnexa ,bone mets

## MDT OUTCOME & PLAN

MDT OUTCOME & PLAN

ZO' ADEX-AI-ZOMETA
Trans vaginal ultrasound to asses adenxae
bone scan and consider palliative RTH according
pain management ,, follow up tumor markers every 3 months

## SIGNATURE

| | | | |
|---|---|---|---|
| MEDICAL ONCOLOGY | MEDICAL ONCOLOGY<br>د/شريف حسانين<br>د/يحيى اسماعيل | RADIOLOGY | RADIOLOGY |
| SURGERY | SURGERY | PATHOLOGY | PATHOLOGY |
| Psychiatric | Psychiatric | Nutrition | Nutrition |

MDT Form
[[DOCTYPE:MDT_FORM]]

| | | | |
|---|---|---|---|
| Patient Name | : الأستاذه / سهير سلامه عبد المجيد احمد | Request No. | : 13003844 |
| Patient No. | : 27902112102367 | Request Date | : 22/08/2024 |
| Path No. | : 3044/24 | Verification Date | : 26/09/2024 |
| Sex | : Female | Financial Category | : [illegible] - نفقة الدولة |
| Age | : 45 Y | Department | : Out Patient |
| Referreing Doc | : | Registered by | : marwa mohamed [illegible] |

## Pathology Report

Clinical Data: Bilateral breast masses
Nature of Specimen: Tru-cut biopsies, collected at 22/8/2024, 10:30 am

Gross Examination:
1- Right breast: Multiple grayish white tissue cores up to 0.7 cm, totally submitted.
2- Left breast: Multiple grayish white tissue cores up to 0.5 cm, totally submitted.
- Sections prepared from paraffin block for immune-staining using Bench Mark ULTRA system for ER, PR, HER2, ER clone (1E2), PR clone (SP1), HER-2 clone (4B5), & Ki67 clone (30-9).

Microscopic Examination of both biopsies:
* Histological type: Invasive duct carcinoma
* Grade (differentiation +nuclear pleomorphism+ mitosis): (3+2+2=7) Grade II
* DCIS: No
* IPT (Positive control showed appropriate reactivity): (On Right Breast)
- E-Cadherin: Positive.
- ER: Strong nuclear reaction in 80% of tumor cells (score 8/8)
- PR: Strong nuclear reaction in 80% of tumor cells (score 8/8)
- Her2: No membrane reaction in tumor cells (score 0)
- Ki67: 20% nuclear reaction of tumor cells
* IPT (Positive control showed appropriate reactivity): (On Left Breast)
- ER: Strong nuclear reaction in 80% of tumor cells (score 8/8)
- PR: Strong nuclear reaction in 80% of tumor cells (score 8/8)
- Her2: Faint incomplete membrane reaction in >10% of tumor cells (score 1+)
- Ki67: 30% nuclear reaction of tumor cells

Diagnosis:
Bilateral breast masses, Tru-cut biopsies:

Verified by :
Dr.Heba Hasan



CAP ACCREDITED
غرفة رقم (4) – الدور الأول – مبنى رقم (987) – كورنيش النيل – مصر القديمة – القاهرة – مصر
Room No. (4) - First Floor-Building No. 987- Nile Corniche - Misr El Qadema - Cairo, Egypt

Pathology Report
[[DOCTYPE:PATHOLOGY_REPORT]]

| | | | |
|---|---|---|---|
| Patient Name | :الاسكندرية / مهير سلامه عبد المجيد احمد | Request No. | : 13003844 |
| Patient No. | :279021121023 67 | Request Date | : 22/08/2024 |
| Path No. | : 3044/24 | Verification Date | :26/09/2024 |
| Sex | : Female | Financial Category | : اوراده نور السلام - بطاقة الخزانة |
| Age | : 45 Y | Department | : Out Patient |
| Refarreing Doc : | | Registered by | : marwa mohamed youss |

## Pathology Report

Invasive duct carcinoma, Grade II
IPT (On Right Breast):
- ER: Positive (score 8/8).
- PR: Positive (score 8/8).
- Her2: Negative (score 0).
- Ki67: Positive in 20% of tumor cells.
IF-T (On Left Breast).
- ER: Positive (score 8/8).
- PR: Positive (score 8/8).
- Her2: Score 1+.
- Ki67: Positive in 20% of tumor cells.

Signature
Dr. Rana Fathy
Dr. Heba Hassan

Verified by :
Dr.Heba Hasan



غرفة رقم (4) - الدور الأول - مبنى رقم (987) - كورنيش النيل - مصر القديمة - القاهرة - مصر
Room No. (4) - First Floor-Building No. 987- Nile Corniche - Misr El Qadema - Cairo, Egypt

Pathology Report
[[DOCTYPE:PATHOLOGY_REPORT]]

Ministry Of Health
Specialized Medical Centers
Dar El Salam Cancer Hospital
Radiology Department

وزارة الصـــــحة والسكـــان
أمانة المراكز الطبية المتخصصة
مستشفى أورام دار السلام
ادارة الاشعة

| | |
|---|---|
| Patient Name: Sohair Salama Abd Al Maguid | Date : 01/10/2024 |
| Exam : C.T CHEST | Age : 45 Year(s) |
| Medical ID: 3375/2024 | Pat. ID: 80337 |

## ***Non CONTRAST MULTI-SLICE CT SCAN OF THE CHEST, ABDOMEN & PELVIS REVEALED: -***

1. ***Chest:***

- Bilateral breast heterogenous parenchymal distortion with diffuse skin thickening .
- Right axillary level I ,II pathologically enlarged lymph nodes the largest measures 3.2x1.2 cm.
- Left axillary level I ,II & III pathologically enlarged lymph nodes the largest measures 2.3x1.3 cm.
- Bilateral prominent subcentimeteric posterior trainagle lymph nodes
- Moderate right pleural effusion.
- Right lower lobe posterior segment tiny pulmonary nodule measuring about 6 mm.
- Right middle lobe consolidative /collapse patchy area with inter
- No pathological hilar or mediastinal lymph nodes.
- Average cardiac size. No pericardial effusion detected.

2. ***Abdomen and pelvis:***

- The liver is **enlarged** with homogenous CT texture. No definite focal lesions or contour irregularities.
- No intra-hepatic or extra-hepatic biliary radicles dilatation.
- Normal size & parenchymal texture of the spleen with no focal lesions seen.
- Normal size, parenchymal thickness & density of both kidneys with no stones or back-pressure changes.
- Smooth filling of the urinary bladder with no filling defects or diverticular outpouching.
- **Mild Ascites with smudging of peritoneal fat and nodular densities , the largest measured 1.6x1 cm seen left hypchondrial region**
- **Bilateral both adnexa measuring 3.2x 5.8 cm on left side & 4x4.2 cm on right side**
- **Mild circumferential gastric wall thickening**

CT Scan
[[DOCTYPE:CT_SCAN]]

| | |
|---|---|
| Patient Name: Sohair Salama Abd Al Maguid | Date : 01/10/2024 |
| Exam : C.T CHEST | Age : 45 Year(s) |
| Medical ID: 3375/2024 | Pat. ID: 80337 |

3. ***The scanned skeleton:***

- **The scanned skeleton shows multiple mixed lytic or sclerotic lesions in axial and appendicular skeleton**

***OPINION:***

- ***Bilateral breast malignant process with pathological bilateral axillary lymph nodes as described.***
- ***Right pulmonary nodule with moderate right pleural effusion.***
- ***Mild ascites with sumding of fat planes and peritoneal nodularity as described .***
- ***Bilateral bulky adnexa for further US /MRI assessment.***
- **The scanned skeleton shows multiple mixed lytic or sclerotic lesions, likely metastatic .**

Dr. Mai Salem

Signature Prof. Menna Tallah Hanafy
15/10/2024 12:37 م

CT Scan
[[DOCTYPE:CT_SCAN]]

?????, ???? **Patient ID: 10110224012914 DOB: 11-Feb-1979 Date: 11-Nov-2024**
Patient Lastname: [redacted]
Patient Firstname: [redacted]
HIS Patient ID: 10110224012914
RIS Patient ID: 361786
Date of Birth: 11/02/1979

Procedure: 70030, Bone Scan
Medical Question:
Clinical Comment:

========== REPORT TEXT ==========
99mTc MDP Bone Scan Report (11/11/2024)

* Whole Body Anterior and Posterior Planar Views Acquired 2Hrs. Post I.V Injection of 20mCi of 99mTc MDP Revealed:

" No definite evidence of active osseous deposits seen all over the surveyed skeleton.
" Degenerative changes over the dorso-lumbar vertebrae.
" Arthritic changes are noted over both shoulder and both knee joints.
" Both kidneys are visualized.
Conclusion: -
" No definite evidence of active metastatic osseous deposits could be seen all over the surveyed skeleton.

Much Obliged
Alaa Elsaeed
Omnia Talaat, MD

========== REPORT TEXT ENDE ==========

Bone Scan
[[DOCTYPE:BONE_SCAN]]